Lapatinib + capecitabine

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1–14 · lapatinib and capecitabine

Active medicines: Lapatinib + Capecitabine

Emetogenic risk: Oral minimal-to-low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine premedication is specified. Choose individual rescue antiemetics only after QT, liver, kidney and interaction review.
- Agree an early diarrhoea plan. NCCP example: loperamide 4 mg initially then 2 mg after each loose stool, maximum 16 mg/day or local policy. Severe diarrhoea or dehydration requires clinical assessment and dose-hold review.
- Capecitabine food timing differs from lapatinib. Pharmacy review is required for acid-suppressing medicines; separation alone does not reliably resolve a PPI interaction.
- Skin / hand-foot care, dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) status, current renal function and current toxicity must be reviewed before a new cycle.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00217 v7 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.16 · Regimen-level drafts; prescriber and oncology-pharmacy verification required